

Referral Protocol — Psychiatrists & Specialists

Document ID	TG-POL-04	Effective	March 1, 2026
Applies to	All Employees (Therapists, Counselors, Coaches)	Owner	Clinical & Compliance Team

At a Glance

Refer for	Symptoms exceeding your scope, possible medication need, client request, escalating symptoms, specialized care Togethr doesn't offer.
Not a referral	Active safety emergencies go to Emergency Protocol (TG-POL-05) instead — do not route a crisis through the standard referral flow.
How	Warm hand-off in session → in-app "Refer to Specialist" flow → document → follow up within 5 business days.
Never	Diagnose outside your scope, delay for caseload reasons, or discourage a client from pursuing psychiatric care.

1. Purpose

This policy describes when and how Togethr employees refer a client to a psychiatrist or other specialist outside the platform's standard coaching and therapy offerings. Referral is a routine, expected part of good clinical practice — it is not a failure to help the client, and no employee should hesitate to refer out of concern for client retention or caseload.

2. When a Referral Is Required

- The client's presentation exceeds the employee's scope of practice as defined in TG-POL-01, TG-POL-02, or TG-POL-03
- The client may benefit from a medication evaluation
- The client requests a psychiatric evaluation or specialist opinion directly
- Symptoms are escalating despite consistent engagement in coaching or therapy
- The presenting concern requires a level of specialized care Togethr does not offer (e.g., inpatient eating disorder treatment, addiction medicine)

Note: an active safety emergency (imminent self-harm, harm to others, or a disclosure requiring mandatory reporting) is handled under Emergency Protocol (TG-POL-05), not this referral process.

3. Types of Specialists and When to Use Each

- Psychiatrist — medication evaluation or management; required when medication may be clinically indicated
- Licensed psychologist — formal diagnostic testing or assessment beyond what a Togethr therapist provides
- Primary care physician — when symptoms may have a physical health cause (e.g., fatigue, appetite change)
- Addiction medicine specialist — for suspected substance use disorders
- Eating disorder specialist team — for any disordered eating with medical risk indicators
- Domestic violence advocate / legal aid resource — for clients disclosing abuse who need resources beyond clinical support

4. The Referral Pathway

Referrals should be a warm hand-off wherever possible, not a cold referral. Follow these steps:

- Discuss the referral directly and compassionately with the client during session — frame it as an addition to their care, not a replacement or rejection
- Use the in-app "Refer to Specialist" flow to generate 2–3 vetted provider options matched to the client's location and insurance where available
- Document the referral conversation and the options given in the client's file
- Follow up with the client within 5 business days to confirm they were able to schedule, and document the outcome
- Continue supporting the client within your own scope while the specialist referral is pending, unless the presenting issue makes that inappropriate

5. What Not To Do

- Do not attempt to diagnose a condition outside your scope in order to "confirm" the need for referral — describe observable symptoms and let the specialist diagnose
- Do not delay a referral because of caseload, client relationship concerns, or a desire to keep working with the client
- Do not discourage a client from pursuing psychiatric care or medication if they are considering it
- Do not guarantee outcomes or specific providers — offer options, not promises

6. Timeline Expectations

- Referral conversation with client: same session the need is identified

- Provider options generated via in-app flow: immediately following the session
- Follow-up with client to confirm scheduling: within 5 business days
- Documentation of referral and outcome: within 24 hours of the referral conversation

7. Example Scenarios

- A therapist's client has been in treatment for six months for depression with limited improvement; a psychiatric medication evaluation may help. The therapist raises this directly and generates options via the in-app flow.
- A coach's client discloses symptoms suggesting a panic disorder. The coach refers to a Togethr therapist (an internal referral within the platform, using the same warm hand-off principles).
- A client asks their counselor directly for a referral to a psychiatrist because they're curious about medication. The counselor should support this request rather than talk the client out of it.

8. Frequently Asked Questions

Q: Is a referral to another provider on the Togethr platform (e.g., Coach to Therapist) handled the same way as an external referral?

A: Yes — the same warm hand-off principles apply, using the in-app flow, even when the referral stays within Togethr.

Q: What if the client refuses the referral?

A: Document the conversation and the client's decision. Continue supporting the client within your own scope, and revisit the referral conversation periodically if the concern persists.

Q: Who selects the vetted provider options shown in the in-app flow?

A: The Clinical & Compliance Team maintains and vets the specialist directory used by the in-app referral flow.

9. Related Policies

This document should be read alongside the scope-of-practice policy for your role (TG-POL-01, TG-POL-02, or TG-POL-03) and Emergency Protocol (TG-POL-05) for situations that require immediate escalation rather than a standard referral.